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Relational Psychoanalysis: A Review

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*Relational psychoanalysis, a relatively new and evolving school of psychoanalytic thought, is considered by its founders to represent a “paradigm shift” in psychoanalysis. The relational approach, initiated by the publication of Jay Greenberg and Stephen Mitchell’s book, *Object Relations in Psychoanalytic Theory* in 1983, has developed into a movement with its own substantially separate literature. This paper reviews both the history and theoretical origins of the relational movement, as well as important theoretical premises and viewpoints now associated with the relational school.*

KEYWORDS *Relational psychoanalysis, Stephen Mitchell, object relations theory, self psychology, infant research, postmodernism*

INTRODUCTION

Relational psychoanalysis is a relatively new and evolving school of psychoanalytic thought. Considered by its founders to represent a “paradigm shift” in psychoanalysis, the relational approach was initiated by the publication of Jay Greenberg and Stephen Mitchell’s book, *Object Relations in Psychoanalytic Theory* in 1983. This work, and the relational movement that followed it, brought together various strains of psychoanalytic theorizing, all of which were seen to assign primary importance to real interpersonal relations, rather than to instinctual drives, in their understanding of human motivation and personality. The relational approach thus began as a work of “selective integration” of compatible psychoanalytic models and approaches, particularly the interpersonal school of Harry Stack Sullivan (the psychoanalytic tradition in which both Mitchell and Greenberg were educated), British object relations theory, and the self-psychological theory of Heinz Kohut, among others. Each of these schools of thought were seen to be conceptually rooted

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in a “relational” premise that differed from the fundamental “drive” premise of traditional Freudian psychology.

Greenberg and Mitchell characterized all these theoretical systems as “relational models,” creating a both a theoretical umbrella and, eventually, an organizational context for dialogue and collaboration among “relational” theorists and practitioners from otherwise differing backgrounds. It should be evident in this context that relational psychoanalysis did not originate as a single school of thought and cannot therefore be neatly summarized by a comprehensive set of theoretical propositions to which all relational psychoanalysts subscribe. Relational psychoanalysis refers to a spectrum of psychoanalytic theories and theorists, brought together on the basis of a common set of fundamental premises about human nature. These schools of thought all diverge from traditional Freudian conceptualizations of human motivation and the nature of the mind. Some of the theorists brought together under the “relational” rubric, such as Harry Stack Sullivan and Ronald Fairbairn, were explicit in the criticism of Freud’s drive theory. Others, such as Donald Winnicott and Hans Loewald, fit their novel ideas into a traditional psychoanalytic framework, utilizing “drive” terminology to express ideas that, on a relational construal, actually differed from those of Freud.

The relational movement gained its first institutional foothold when it became a separate (and among candidates studying in the Program, an instantly popular) official “orientation” within the New York University Post-doctoral Program in Psychotherapy and Psychoanalysis, in 1988. In 1991, a scholarly journal devoted to development of the relational school of thought was launched under the editorship of Stephen Mitchell. *Psychoanalytic Dialogues: A Journal of Relational Perspectives* immediately became a well-respected and influential publication, attracting submissions from around the world. Relational ideas quickly grew in influence, especially in New York, and have had a worldwide influence in countries including Italy and other European countries, in Israel, and elsewhere.

The most influential relational psychoanalyst has been Stephen Mitchell, whose writings provide the primary, though not the only, source for this brief overview of relational theory. Between the time he published his 1983 volume with Jay Greenberg and his sudden and untimely death in December 2000, Stephen Mitchell authored or co-authored several subsequent, widely read books (e.g., Mitchell, 1988, 1993a, 1997, 2000). Mitchell’s account of relational theory is the primary source for this overview. His enormous output following the publication of his book with Greenberg, along with the contributions of a growing circle of relational psychoanalysts, transformed the relational model from a product of selective integration into a vibrant international movement, producing an exciting and original literature, nurtured by developments in research and adjacent schools of thought (e.g., attachment research, intersubjectivity, mentalization), and giving rise to new training institutes and professional psychoanalytic organizations.

THE RELATIONAL CHALLENGE TO DRIVE THEORY

The core challenge of relational theory has, from its inception, been directed toward Freudian “drive theory” and the “drive/structure” model of mental life posited by Freud (Greenberg & Mitchell, 1983). In order to articulate the relational objections of Freudian theory, it is necessary to outline those tenets of Freudian theory that are the subject matter of their critique. In the following paragraphs, then, we review these features of Freud’s theorizing as they are seen by their relational critics. The rendition is accurate in so far as it goes, but it is selective, emphasizing only those features that are relevant to the relational challenge (namely the mechanistic and biological components of the theory), and omitting other aspects, which are outside the scope of this focus.

From its inception, psychoanalytic theory concerned itself with the motivations that underlie mental life and behavior. Freud conceptualized the primary motivations that “drive” human behavior as “instinctual drives,” formed by the phylogenetic experiences of the human race, and rooted in the elemental constitution of every human being (Freud, 1905). Early in his theorizing, Freud construed the primary motivations as the sexual and self-preservative drives, often epitomized as “sex and hunger” (1905, 1915). Some years later, Freud (1920) linked both these drives within a larger notion of “Eros” or the life instinct. At the same time, he posited a “death instinct” which, in his view, gave rise to an aggressive drive. While many analysts rejected the idea of a death instinct (e.g., Simmel, 1944), virtually all classical analysts since Freud have recognized both sex and aggression as important motivations in mental life, a notion often characterized as the dual-drive theory (e.g., Hartmann, Kris, & Loewenstein, 1949).

In Freud’s formulations, instinctual drives have their source in the body but are represented in the mind as tension states, or states of “unpleasure” that can only be relieved through an action that satisfies the instinctual demand (Freud, 1905, 1915). If sexual needs, originating in the sex organs, produce a tension state in the mind, only a sexual action (e.g., an action leading to orgasm) will dissipate that tension. Such action, leading to “drive discharge,” produces a subjective feeling of relief, or “pleasure.” Patterns of successful drive discharge (actions that reduce unpleasure) are represented in the mind as “wishes,” or more broadly, as “desire.” Because sexual (and often aggressive drives, as well) require the involvement of another person (formally characterized as the drive’s “object,” in Freudian terminology), drive discharge motivates interpersonal events and engagements. Interpersonal relations are thus seen as “drive derivatives.”

Instinctual drives were seen by Freud as perpetually pressing the individual to action in the service of drive discharge, the intensity of the pressure they exert increasing or decreasing in accordance with the biological processes at their source (Freud, 1915). The realities of social life, however,

make it impossible for anyone to gratify all their wishes all the time. As a result, patterns of drive discharge must be fashioned to fit a social context. The foregoing view may be summarized by observing that human behavior is orchestrated by drives that are modified by the constraints imposed by the social world. In the relational view, these social constraints, derived from interpersonal relationships, were far less important to Freud and his followers than the drives they constrained.

Relational theorists challenge this view of human motivation, decisively rejecting the idea that human beings seek out interpersonal relationships for the primary purpose of gratifying instinctual sexual or aggressive drives. A cornerstone of all relational theory is the premise that human beings are born with a primary need for relatedness and communication with other human beings. This need is seen as fundamental to the human organism, and its satisfaction is held to be necessary for normal development and survival. In the relational perspective, then, social experience is primary as a motivation and as an organizer of mental life. It is social experience—not any biological, preformed instinctual drives—that shapes our personalities.

Evidence for the existence of a primary need for relationships is drawn from multiple sources. Children who are abused or neglected by their parents are seen to be intensely attached to their hurtful and frustrating parents, despite the fact that these relationships are a source of suffering, rather than of pleasure. Similarly, adults may seek out relationships with others that repeat traumatic injuries and frustrations. Traditional analytic theory has explained such self-injurious behavior by reference to faulty ego functioning, primitive defenses, latent sexual meanings and pleasures embedded in painful experience, the repetition compulsion, and the death instinct. Relational theorists, on the other hand, who do not regard all behavior as motivated by the pursuit of pleasure, can readily explain such maladaptive attachments as satisfying a primary need to maintain human relatedness and preserve or repeat important relationships, despite the pain these relationships entail.

Relational theorists also cite very substantial evidence from attachment research (Bowlby, 1969, 1973, 1975) as well as infant research (Lichtenberg, 1983) that points to the primacy of a need for interpersonal relationships. Reviewing a large body of such research, Lichtenberg (cited by Mitchell, 1988, p. 24) writes, “Study after study documents the neonate’s preadapted potential for direct interaction—human to human—with the mother.” Daniel Stern (2005) points to the existence of “mirror neurons” as evidence that human beings are innately equipped to understand one another’s experience and, thus, to relate to one another in a psychologically meaningful way (p. 80). Mirror neurons are neurons situated in the motor cortex that fire when another is observed in various kinds of behavior. Notably, their pattern of firing replicates the neural activity that would occur if the observer were engaged in the same behavior. Mirror neurons are thus seen as an innate neural foundation for our empathic understanding of the actions and intentions of

others. Iacoboni (2008) suggests that the absence of sufficient motor neurons is the cause of autism, whose pathognomonic feature is “mindblindness,” or the inability to understand the minds of others.

The relational claim that human beings seek connection with others as an end in itself, rather than as a means to gratify instinctual drives, accords with contemporary ideas about the evolution of human beings. As early social life became more complex, natural selection would have favored those of our ancestors who were capable of forming good relationships with others and negotiating the demands of social life successfully. As cultures become increasingly complex, greater social intelligence is needed to survive. “In a sense,” writes anthropologist Clifford Geertz, “the brain was selected by culture” (quoted in Mitchell, 1988, p. 18). The relational view is also supported by ethological studies, such as those cited by Bowlby, who construed attachment behavior as ensuring the proximity of the mother to the child, and thus enhancing the child’s likelihood of survival (Bowlby, 1969). “The infant,” Mitchell concludes, “does not become social through learning or through conditioning, or through an adaptation to reality . . . the infant is programmed to be social. Relatedness is not a means to some other end . . . the very nature of the infant draws him into relationship. In fact, relatedness seems to be rewarding in itself” (p. 24).

RELATIONAL CONFIGURATIONS AND THE NATURE OF THE MIND

As just described, human beings are motivated by an innate desire to establish interpersonal relationships with others. The character of the relationships we seek to form, as well as those we seek to avoid, are determined not by preformed instinctual patterns but by actual experiences with other people. Early experiences with parenting figures give rise to expectations about how we will be treated by others. These expectations, in turn, motivate our subsequent relational strivings. Our interpersonal experiences thus give rise to the motivations that will impel us to further relationships. Mitchell and other relational theorists view the mind as composed not of instinctual drives, but of “relational configurations” (Mitchell, 1988, p. 3).

Relational configurations are models of interpersonal or “relational” experiences that we seek with others as well as those painful or frightening experiences that we seek to avoid. As previously noted, relational configurations are formed on the basis of actual life experiences, but they are not necessarily veridical representations of those experiences, as all encounters of the objective world are experienced through a prism of representations and fantasies, formed on the basis of yet earlier experience. Like life experiences themselves, relational configurations are posited to include three dimensions: the self, the object, and the psychological “space” in which the two interact. “These dimensions are subtly interwoven, knitting together

the analysand's subjective experience and psychological world" (Mitchell, 1988, p. 33). In the relational model, interpersonal experience and the relational configurations to which they give rise are the building blocks of the mind. Insofar as the mind is structured through interactions with others, it is impossible to conceive of a mind in isolation, outside of a context of interpersonal relationships. Relationships are the "stuff" of the mind, Mitchell writes.

Relational configurations determine our perceptions of others, our expectations of their attitudes and responses to us, and our reactions and behavior toward them. Although relational configurations are stable and repeated, producing a feeling of continuity and consistency of life experience, they are also normally subject to the influence of new experiences with others. Thus, while relational configurations shape our experience of others, they are, in turn, also shaped by our actual interpersonal experiences through life. The potential openness of relational configurations to change on the basis of new interpersonal experience is fundamental to the relational view of psychotherapeutic treatment.

Relational configurations also shape our experience of ourselves. Relational theorists do not view the "self" as a unified structure located outside a social or relational field. The self is a network of impressions that we form of ourselves in the context of our relationships with others. Harry Stack Sullivan wrote that the self is formed, at least in part, by the "reflected appraisals" of others (Sullivan, 1940, p. 22). Kohut, writing many years later, would similarly refer to the importance of "mirroring" in the development of the self (Kohut, 1971, 1977). For Sullivan, Kohut, and the relational theorists who followed, the notion of "self" is only meaningful within the context of our relationships with other people, including both current interpersonal relationships in the real world as well as those we preserve within our memories and imaginations.

Psychopathology in the relational perspective is characterized by relational configurations that are conducive to painful relationships, constrictions in our capacity for authenticity, mutual intimacy and love, and impairments in the quality of self-experience. Although relational configurations are normally susceptible to transformation in the context of new experience, this mutability is more limited in psychopathological conditions. Psychopathology is characterized by the maladaptive character of relational configurations, by the rigidity of those configurations, and, importantly, by persistent attachments to painful or frustrating relationships with primary figures (as in the case of abused children, noted previously). As will be seen in the following paragraphs, the clinical practice of relational psychoanalytic therapy features not only the creation of novel and more adaptive relational configurations through the therapeutic interaction, but also entails, to one degree or another, the elucidation and interpretation of those persistent relational configurations that limit the client's relational potential and happiness.

SEX AND AGGRESSION IN RELATIONAL THOUGHT

Although the relational model is a vigorously “environmental” model of personality functioning, positing that personality organization is structured by interactions with others, it is not, on that account, exclusively or “naively” environmental. As discussed earlier, our motivation for human interaction as well as our pre-adapted capacity for complex human relationships can both be traced to evolutionary pressures. In this sense, relational theory is no less Darwinian than Freudian theory. Similarly, although relational theory does not construe sexuality or aggression as “drives,” sex and aggression are both prominent features of human life, and relational theory acknowledges and accounts for both from within its own perspective.

Sexuality

In the Freudian view, sexuality and aggression motivate interpersonal events, the meanings of which derive from their instinctual sources. In the relational view, on the other hand, sexual and aggressive acts are understood as species of interpersonal behavior, reflecting unique personal meanings about the self, the object, and the relationship being enacted between them.

It is commonplace among Freudian analysts and relational analysts, as well as just about everyone else, to regard sex as an experience of intense arousal, excitement, and pleasure, and therefore a prominent feature of human life. Where Freudian theory construes sexuality as the expression of an instinctual drive, rooted in the body, and persistently pressing for discharge, relational theory views sexuality as a biologically generated physiological response, cued by sexual stimuli in an interpersonal context. In non-humans, the cues for sexual arousal are necessarily real, but in humans, with our mental capacities for representation and imaginative elaboration, the cues for sexual arousal may be real, remembered, or imagined. This understanding of sexuality is supported by research in animal sexuality, which is summarized by a noted expert on animal sexuality that is quoted by Holt (1976, p. 173) and cited by Mitchell (1988, p. 82):

To a much greater extent than is true of hunger or thirst, the sexual tendencies depend for their arousal upon external stimuli. The quasi-romantic concept of the rutting stag actively seeking a mate is quite misleading. When he encounters a receptive female, the male animal may or may not become sexually excited, but it is most unlikely that in the absence of erotic stimuli he exists in a constant state of undischarged sexual tensions. This would be equally true for the human male, were it not for the potent effects of symbolic stimuli which he tends to carry with him wherever he goes.

Relational theorists view sexuality as a state of bodily arousal and excitement that is absorbed into a world of psychological meanings in accordance with the relational contexts in which they emerge. Because sexual excitement occurs early in life in the context of relationships with primary caregivers, it takes on meanings that derive from the nexus of relational experiences that characterize infancy and early childhood. Mitchell cites Melanie Klein (1945, 1957), who believed that sexuality emerges in the developmental context of the “depressive position,” and therefore accrues meanings in the vortex of infantile aggression, depressive anxiety, and reparative strivings. In this relational context, giving and receiving sexual pleasure takes on meanings related to repairing the mother, providing pleasure rather than pain, and replacing depressive affects with excitement and joy (Mitchell, 1988, pp. 95–96).

Set in different relational contexts, sexuality takes on an infinite variety of meanings. All of these meanings are relational in character. Some pertain to the relationship with the object (for example, penetrating the mystery of parent’s secret life, securing the full and intense engagement of an elusive caretaking other, acquiring privileged access to the privacy of an opaque other, passively receiving the caretaking of the other, surrendering to a powerful other, achieving power over the hurtful other, healing a damaged other, and so on). Some of these purposes entail the use of sexuality in the maintenance and support of the sense of self. Kohut (1971, 1977) and other self psychologists (Stolorow & Lachmann, 1980) have described how narcissistically impaired individuals can seek sexual encounters in order to bolster a sense of self as attractive, as masculine or feminine, as adequate in their gender identifications and capable of being desired. The intense arousal of sexuality can serve to support a sense of vitality, personal agency, and, in some cases, personal existence.

These meanings, encoded in the individual’s relational configurations, give rise to sexual behavior throughout the course of life, not as a means to achieve discharge of a putative sexual instinct, but as a means to integrate interpersonal contexts in which relational needs can be enacted and expressed. There is no such thing as sexuality without relational meanings. Even the individual masturbating in solitude is intimately engaged in relational activity, albeit in his or her imagination rather than in real life.

Aggression

Freud came to view aggression as a drive late in his theorizing, at first having considered it to be a component of the sexual instinct, equipping the individual to actively pursue and gratify sexual purposes (Freud, 1905), and somewhat later, viewing aggression as a component of the self-preservative instinct (1915). In these renditions, aggression subserves sexuality and self-preservation but does not exert a demand of its own. In 1920, as previously

noted, Freud posited that aggression is a derivative of the death instinct and a determinant of a wide spectrum of psychopathological conditions, such as depression and sadomasochistic perversions and character pathologies, as well as the manifest human destructiveness of war. While some analysts, notably Melanie Klein, adopted the notion of a death instinct and its primordial destructiveness, other psychoanalysts rejected the notion, preferring to conceptualize aggression as an instinctual drive in its own right, on a par with the sexual instinct (albeit lacking a specifiable bodily source).

Many analysts have also come to reject this formulation, positing that aggression is a reaction to situational triggers, rather than a drive that perpetually needs to be gratified. In this view, which characterizes relational thinking, aggression is a biologically programmed pattern of physiological arousal that readies a person for action in situations of endangerment or frustration. In his review, Mitchell (1993b) cites Sullivan (1956), who believed that aggression was triggered by situations of helplessness and anxiety; Fairbairn (1952), who saw aggression as triggered by frustration of dependency needs and object seeking; Guntrip (1969), who viewed aggression as a reaction to anxiety and “ego-weakness;” and Kohut (1977), who understood aggression as a reaction to disruptions in needed self-object ties. Mitchell quotes Fromm’s conclusion that *“phylogenetically programmed aggression, as it exists in animals and man, is a biologically adaptive, defensive reaction”* (1993b, p. 358, italics original). This understanding of aggression, Mitchell writes, is consistent both with contemporary ethological views (pp. 363–364) as well as with the observational research of Parens (1979), which convincingly demonstrates that aggression is a reaction to unpleasure and has as its goal the elimination of that unpleasure (p. 366). Aggression in relational theory, then, is seen as a biologically prewired response to environmental triggers rather than a product of instinctual drives.

In Mitchell’s rendition, as in much of the relational literature, aggression is triggered by a sense of endangerment, not only in the sense of threats to one’s physical well-being but also to one’s sense of self. Helplessness and anxiety, frustration of dependency needs and object-seeking, experiences of “ego-weakness,” the failure of others to validate one’s self, and protracted or intense experiences of pain or unpleasure are all experienced as endangering a person’s well-being. “The universality of some experiences of endangerment in infancy and early childhood leads to the inevitable dynamic centrality of aggression” (p. 367).

Although aggression is not a drive, writes Mitchell, it can appear to be a drive when experiences of emotional hurt, frustration, and endangerment are absorbed into an individual’s relational configurations, giving rise to expectations of hurt or frustration, perceptions of others as hostile or hurtful, and feelings of narcissistic injury, all of which arouse aggression and hostility. In this rendition, aggression is triggered, not only by current interpersonal events that are actually endangering, but also by remembered

events, anticipated events, and benign events that are subjectively construed as endangerments according to an individual's relational configurations. The more prominent and pervasive the experience of endangerment in a person's relational configurations, the more that aggression takes on the appearance of a drive. "To regard aggression as a reaction does not necessarily minimize its motivational or structural primacy" (p. 369).

Mitchell's formulations about aggression facilitate the integration of two opposing positions with regard to how aggression is viewed in theory and addressed in practice. When viewed from a classical Freudian perspective as an instinctual drive, aggression and the behavior it motivates are seen to be unjustified, an exploitation of other persons and interpersonal events for the purposes of drive discharge. From the aggressive person's point of view, however, his or her aggression is justified by the hurtful or threatening conduct of others (that is, by real situations of endangerment). From a non-drive point of view that construes aggression as a reaction to threats in the immediate environment, aggression is always justified because aggression is a natural reaction to endangerment. This dichotomized attitude toward aggression manifests opposite reactions to the client's aggression within the analytic situation. If the therapist views the client's aggression as unjustified, the client will inevitably experience this statement as false, producing a disruption in the client's experience of the analyst as understanding his or her experience. If, on the other hand, the therapist views the client's aggression as a justified reaction, the client's contribution to his own experience of endangerment is apt to be overlooked. Mitchell's relational formulation provides a more comprehensive account, allowing both the client and analyst to look at the actualities of the interpersonal situations that arouse aggression, thus maintaining empathic contact with the client, while also enabling the client to differentiate between those situations and his or her subjective elaborations and construals of those situations.

THEORETICAL ROOTS AND DEVELOPMENTS OF RELATIONAL IDEAS

Although relational psychoanalysis was organized as an umbrella movement, "American interpersonal" psychoanalysis, as propounded by Harry Stack Sullivan, has clearly been the dominant theoretical influence on its development. British Object Relations theory, as represented especially by the writings of Ronald Fairbairn and Donald Winnicott, has also been very influential, as have the work of infant researchers, postmodern philosophy, and, to some extent, the writings of Heinz Kohut. Sándor Ferenczi, one of the first generation of psychoanalysts following Freud, has also come to be appreciated as a pioneer who introduced many ideas and clinical approaches later adopted and developed by relational psychoanalysts.

American Interpersonal Psychoanalysis

Harry Stack Sullivan, the originator of American interpersonal psychoanalysis, was a staunch critic of classical psychoanalytic theory and treatment techniques. Sullivan approached psychotherapy in an operational, pragmatic way. As described elsewhere in this paper, Sullivan believed that personality is organized through interpersonal experiences with “significant others.” These interpersonal experiences are integrated over and over throughout development as the child pursues the satisfaction of his or her needs within the context of his or her needs for security and self-esteem. Sullivan understood both anxiety and security as states of being that are mediated by interpersonal relations. In his view, anxiety is elicited by the disapproval of significant others, while security and self-esteem are produced when significant others demonstrate their approval. On the basis of his or her interpersonal experiences with significant others, every individual develops internalized personifications of himself or herself as “good-me,” “bad-me,” and—in consequence of experiences of extreme anxiety—“not-me.” In addition, Sullivan conceptualized a “self-system” that serves to maintain self-esteem by “selective inattention”: selectively excluding perceptions that would stir anxiety and experiences of “bad-me” or “not-me” (Sullivan, 1953).

The idea that the personality derives from, and reflects, actual interpersonal relationships led Mitchell (1988) to describe the mind as composed of internalized “relational configurations.” The fact that every child has had more than one formative interpersonal relationship, even with a single person, has led to the idea that all people have a “multiplicity of selves” and, accordingly, that the experience of a unitary personality is fundamentally an illusion. The notion of multiple selves has also led relational theorists to take great interest in dissociation as a basic mode by which the personality is structured. In this view, personality is understood as essentially discontinuous rather than as integrated, not only when subject to defensively motivated defenses, such as dissociation (Bromberg, 2001, 2006; Mitchell, 1993a). As will be seen, this conception is consistent with postmodern thinking (discussed later).

Following Sullivan, Edward Tauber (Tauber & Green, 1959) and, later, Edgar Levenson (1972) and others explored the ways in which clients influence analysts’ inner experience and behavior on an unconscious basis—ideas in some ways similar to later developments of the Kleinian concept of projective identification, which has also come to be of interest to relational analysts. Another interpersonal psychoanalyst, Benjamin Wolstein, described how the unconscious personality dynamics of client and analyst interact in defensive ways (Wolstein, 1954), thus further extending the interpersonal (and later, the relational) understanding to the psychoanalytic encounter.

Erich Fromm (1941, 1947, 1964) was an important contributor to the interpersonal school. His thinking also has been influential in the development

of the relational school of thought. Some of his ideas, notably his advocacy of personal authenticity and freedom rather than conformity, and the corresponding value he placed on authenticity, genuineness, and honesty in the analytic relationship (including confrontation of maladaptive behavior), have also been formative in the thinking of many relational analysts. In the tradition of Fromm's social activism and the interpersonal emphasis on the role of social influences, some relational analysts, most notably Neil Altman (1995), have broadened their interest to the interface between culture and psychology, and developed conceptions to facilitate the extension of psychoanalytic treatment to populations who are not typically treated in psychoanalysis or psychoanalytic therapy.

British Object Relations Theories

British object relations theorists have devoted great attention to pre-oedipal narcissistic pathology and, correspondingly, to the early developmental processes that contribute to the formation of an autonomous, integrated, and well-regulated personality. Fairbairn (1952) famously observed that libido seeks objects (that is, people), not pleasure. Pleasure, he wrote, is a "sign-post to the object," and is pursued for the purpose of integrating object relations. Our most basic needs, he contended, are to be loved and to have our love accepted by other people. On the basis of conflicts that arise in early relationships with caregivers—that is, by the failure of real objects to meet the child's basic needs—Fairbairn mapped out an inner world of various split-off partial "egos" (today we would say "selves") and objects that structure the personality and psychopathology. Mitchell saw Fairbairn's work as compatible with Sullivan's emphasis on the influence of actual interpersonal relations, while adding a more fully developed understanding of the internal world that results from interpersonal influence.

Donald Winnicott (1958, 1965, 1971), another prominent object relational theorist of the British school, questioned how a human being comes to feel like a real, integrated, and separate person, and also how a person comes to know others as persons like oneself; that is, as centers of subjective experience rather than merely as two-dimensional, need-satisfying objects who are experienced essentially as projections. Winnicott's ideas about how children get to know others as real people have been an important influence on relational theorizing, particularly on the work of the influential relational analyst Jessica Benjamin (1988, 1990, 2006). Drawing on the work of Winnicott, and also, notably, on the writings of the German idealist philosopher Hegel, Benjamin has developed a notion of "intersubjectivity" based on "mutual recognition." Intersubjectivity, a topic of great interest to analysts working in relational models, and to which we will return, refers to the capacity to enter into the subjective experience of another. In her writings,

Benjamin emphasizes that people need others to recognize them as “centers of subjectivity,” but must be able to recognize others as “centers of subjectivity” as well if the experience of being recognized by others is to be meaningful. Mutual recognition, however, is an inherently vulnerable state, which may easily collapse into complimentary “doer-done to” relationships that are rooted in sadomasochistic solutions to the need for recognition.

Winnicott’s (1971) concept of transitional phenomena is another idea that has become prominent in relational thought. Winnicott posits an area of experience that belongs neither to oneself nor to the outside world, but that exists “in between”—an area of the outer world that the child (or client) can in some sense feel as a personal possession and to which he or she can attribute meaning without being challenged. This “area of illusion” must be tolerated by caretakers if the child’s (or client’s) discovery of true self and one’s inner creative potential is to take place.

In this tradition of thinking of an important space in between client and analyst, as well as in the vein of ideas about continuous mutual influence between client and analyst (to be discussed), interest has increased among relational analysts in what has been called the “analytic third.” While this notion has been conceptualized in different ways, one important way of understanding “the analytic third” is as a “third subject of analysis,” the other two subjects being the individual subjectivities of analyst and client. The “analytic third” is seen to be a kind of subjectivity in its own right, unconsciously generated by the subjectivities of client and analyst, which takes on a life of its own, changes the analyst’s experience of the client, and provides a platform for the analyst to think about and speak to the client in new ways (Ogden, 2004).

Kohut’s Self Psychology

Kohut (1971, 1977, 1984), the originator of “self psychology,” is a thinker whose ideas are closely related to those of object relations theorists. In Kohut’s theorizing, as in object relations theory, preeminence is given to pre-oedipal needs, and notably, to those experiences that are necessary for the development of a self. Kohut held that the satisfaction of these critical needs comes about only under certain object relational conditions. Central to these conditions is the presence of an empathically attuned caretaker, whose understanding and availability permits him or her to be utilized by the child as “self-object.” The term “self-object” was introduced by Kohut to refer to persons in relation to whom the child experiences the love, empathic understanding, and responsiveness that enable him or her to form, consolidate, and maintain a valued experience of self.

In Kohut’s view, self-objects support self development when they can be idealized, enabling the child to share in the self-object’s admired

characteristics, and when they register and reflect (“mirror”) aspects of the child’s experience, accomplishments, and valued characteristics, supporting the child’s narcissism or age-appropriate “grandiosity.” In keeping with his idea of “self-objects” and their role in early development, Kohut articulated a new understanding of transference and of the healing potential of the clinical situation. Studying the transferences of narcissistically impaired clients, he discovered that these clients develop “self-object” transferences through which they endeavor to complete developmental processes, derailed or thwarted in childhood, that are necessary for consolidation of a stable and valued self. In later writing, he would come to view self-object transferences as a universal dimension of human relationships, necessary for the maintenance of positive self-experience.

Kohut’s work has been very influential in the clinical thinking of some relational analysts. Jessica Benjamin (2006), for instance, follows Kohut in placing great importance on the therapist’s empathic attunement (“recognition”) to a client’s mental state. Like Kohut, she observes that vicissitudes in the therapist’s empathy can disrupt the client’s feeling of connectedness with the therapist, and with it, the client’s feeling of well-being. Such disruptions can be repaired if the therapist can reestablish empathic contact with the client, including the client’s subjective experience of the disruption and acknowledging the analyst’s own contribution to it. For Benjamin as for Kohut, cycles of disruption and repair are central features in the clinical process.

Sándor Ferenczi

Although Sándor Ferenczi was not a direct major influence in the formation of the relational school, his later work (1929, 1930, 1931, 1932, 1933) was a precursor of many of the ideas that have been of interest to relational psychoanalysts. Contemporary relational analysts have rediscovered Ferenczi and have re-popularized his contributions and integrated some of his ideas into the relational model (Aron & Harris, 1993). Among these ideas is the marked emphasis he placed in his later writings on the importance of real events, and especially of psychological traumas, in pathogenesis and the consequences of trauma on the personality, including dissociation and splitting of the personality, pathological identifications and consequent compulsive accommodation to others, the tendency to regress, the tendency to precocious ego development and adoption of a caretaker role, and the tendency to become rigid and mistrustful (see Frankel, 1998, 2002). Ferenczi’s later approach to psychotherapy was pioneering and, within the Freudian tradition, heretical. Ferenczi was the first analyst to propose that the therapist should take on a reparative, parental role with clients who had been traumatized. He was also the first analyst to suggest that an ongoing unconscious dialogue existed between client and analyst or to appreciate the great

extent to which the therapist's countertransference can influence the client and the course of the treatment. Most remarkable in his later work was his experimental adoption of a more mutually expressive form of analysis in order to counteract the negative effects of uncontained countertransference. The important work by relational analysts Davies and Frawley (1994) on the consequences and treatment of psychological trauma can be seen, in some respects, as within the tradition of Ferenczi's ideas about trauma.

Infant Research and Its Implications

Relational psychoanalysis has been very significantly influenced by the work of infant researchers such as Daniel Stern (1985) and Beatrice Beebe, both of whom have developed ideas about the ways in which representations of self, internalized expectations of others, and interaction structures ("representations of interactions that have been generalized," or RIGs, in Daniel Stern's work) are constructed by infants through their early interaction with their mothers. Beebe and Lachmann, her longtime collaborator, have proposed three principles by which early interactions between baby and caregiver influence personality: (1) through repeated, ongoing patterns of interaction; (2) through disruptions and subsequent repairs of attunement; and (3) in moments of heightened affect (Beebe & Lachmann, 1994). (As previously noted, the notion of disruption and repair originally came into the analytic literature through Kohut [1971].) This body of work documents the degree of continuous mutual influence and regulation occurring between people outside of their awareness, and thus provides an empirical and conceptual foundation for the close attention paid by relational analysts to the interaction between client and therapist and to the enactments that are co-constructed by both (Hoffman, 1998).

Infant research has also illuminated the phenomenon of "intersubjectivity," a topic studied across a wide spectrum of disciplines, including developmental psychology, neuroscience, philosophy, and, under the related heading of "theory of mind," among ethologists as well. Interest in intersubjectivity among relational analysts was initially stimulated by Jessica Benjamin (1988, 1990), whose work was inspired by Winnicott and Hegel, and informed by clinical data (see previous discussion of "British object relations"). Daniel Stern's observational study of infants and their mothers (1985) has expanded understanding of intersubjectivity as a discrete and progressive psychological attainment.

Stern (2005) defines intersubjectivity as "the capacity to share, know, understand, empathize with, feel, participate in, resonate with, and enter into the lived subjective experience of another" (p. 78). Stern (2005) posits that intersubjectivity should be conceptualized as a distinct motivational system, along with sexuality and attachment. A primary need for psychological

intimacy, to know and be known by others at the subjective level, is supported by observations that it is a universal tendency among humans and that it favors species survival. Survival of the species requires group formation, group cohesion, cooperation, and coordination in collective action. Morality, a major psychological factor in the preservation of social life, can only arise on the foundations of intersubjectivity. "The 'moral emotions' (shame, guilt, embarrassment) arise from being able to see yourself in the eyes of another, that is, as you sense the other sees you" (Stern, 2005, pp. 83–84). Intersubjectivity also plays an essential role in the development of reflective consciousness, which, like morality, arises on the basis of identifications with the consciousness of others. Intersubjectivity is experienced subjectively as a pressing need for "orientation"; that is, to know the feelings and intentions of the other, to understand the interpersonal events in which we are engaged, and to ascertain the status of interpersonal relationships. Intersubjectivity is also experienced as a felt need in the context of efforts to establish or preserve a sense of personal identity. "We need the eyes of others to form and hold ourselves together" (Stern, 2003, p. 84).

Postmodern Philosophy

Postmodern philosophical trends, represented by writers such as Lyotard, Derrida, Rorty, and Berger and Luckmann, have also had a great influence on relational psychoanalysts. Postmodern thinking, as applied to psychoanalysis, questions whether people have an essence and a unity apart from the diverse and contradictory social influences to which they are subject. Although a radical version of anti-essentialism has never been popular among relational analysts, postmodern thinking has been very influential among relational psychoanalysts. Postmodern theory brings into question our ability to know others as they are, because we can only perceive others from our own limited, biased, and self-interested viewpoint. This point of view is called "perspectivism." At the same time, analysts influenced by postmodern thought question the possibility of observing another person's mental life without simultaneously changing it through the interpersonal influences exerted throughout the endeavor.

Positing that the analyst views the client from within a limited perspective with inescapable perceptual biases, while the client is subject to the unavoidable influence of the analyst's pattern of engagement with him or her, many relational theorists hold that objective knowledge of a client is impossible. We can never get to know the client's inner life in a pristine way, apart from our influence upon it. Postmodern thought thus undermines the credibility of the analyst as an objective observer of the client's subjectivity. Many relational theorists have responded to this challenge by endorsing a pragmatic approach to understanding the client's mental life, questioning for

each client, “Which way of understanding, from among the many perspectives that can account for the clinical data, will best help the client live a fuller, richer life?”

Postmodern thought also inspires relational analysts toward the notion that the analytic relationship and all that transpires within it is inherently “co-constructed” by client and analyst (a notion promoted by Irwin Hoffman, 1998, who conceived of the analytic situation as a product of “dialectical constructivism”). In the relational view, the meanings attributed to the client’s behavior and inner experience are implicitly negotiated between client and analyst rather than being fixed, preexisting, and potentially discoverable “as is” (Pizer, 1998; Mitchell, 1993a). Against the background of the idea that the analytic relationship is inherently co-constructed, Anthony Bass (2007) has discussed the importance of analyst and client negotiating the analytic frame in each case.

One clinical implication of these ideas about the analyst’s unavoidable influence and inescapable biases is that the analyst may at times need to disclose his own perceptions, thoughts, and feelings to the client if the interpersonal field of which the client is an inextricable part, and thus the client her- or himself, is to be most adequately understood. Aron (1996, especially Chapter 8), Renik (1999), and others have developed conceptions of more mutually expressive frameworks of analytic treatment precisely for this reason. Other relational analysts have seen a role for analyst self-disclosure, but for more limited purposes, such as helping the client to question rigidly held convictions about the therapeutic relationship by disclosing the actual impact he or she has on the analyst (Davies, 1994), or repairing impasses by acknowledging the analyst’s role in creating them and thus restoring the client’s trust (Benjamin, 2006). Hoffman (1998) sees analyst self-disclosure as likely to have greater therapeutic impact against an ongoing backdrop of analytic reticence and ritual.

The postmodern notion that the analyst’s influence on the client is unavoidable also raises doubts about traditional principles of analytic neutrality and anonymity. In the face of doubts about whether these conditions can ever be approximated, let alone fully achieved, many relational analysts have taken a much more favorable attitude toward the analyst’s spontaneity and openness in the analytic relationship. Some relational analysts participate more or less freely in mutual enactments with the client—these enactments are seen as unconsciously determined primarily by the client, but also by the analyst in some unconsciously mutually coordinated way—rather than attempting to restrain such participation as much as possible. The rationale for such freedom is the view that, through such mutual enactments, the client’s inner life becomes actualized in a vivid way and thus becomes accessible to reflection, dialogue, and renegotiation. This view is markedly in contrast to the classical idea that the client’s unconscious fantasy can be seen most clearly and experienced most immediately precisely because the

analyst, adopting a position of abstinence regarding the client's transference wishes, does not participate in or gratify the client's transference fantasies and enactments.

BASIC CLINICAL TENETS OF RELATIONAL PSYCHOANALYSIS

Relational psychoanalysis is not a unified theory and contains no unitary theory of psychoanalytic technique. Because the relational school includes practitioners from a wide diversity of backgrounds, each with its own treatment traditions, it is impossible to articulate a single approach that characterizes all. Sullivanian interpersonalists are apt to be more active in their approach than object-relations therapists or self psychologists, for example. Some are more freewheeling, others less so. Some challenge their clients, insistently confronting rigid schemas, while others take an attitude of passive receptivity or empathic immersion.

In general, however, analysts and therapists committed to a relational approach engage the therapeutic situation with a few clinical propositions in mind: (1) personality is formed through interpersonal interactions; (2) the clinical situation inherently intersubjective and shaped by mutual influence must be conducted in a way that incorporates this fact; and, (3) the client needs some kind of authentic personal engagement from the therapist. These assumptions suggest a "model relational approach," characterized, to one degree or another, by the following features:

1. The relationship between client and therapist typically takes "center stage," in part, as a source of information about the client's subjective life, but, more importantly, as an interpersonal context within which habitual relational configurations are transformed, characteristic relational patterns are renegotiated, rigid expectations are called into question, and new relational potentials are actualized;
2. The therapist seeks to understand and articulate the interpersonal field as a whole, including the ongoing and resonating impact of each participant upon the other, rather than observing the client as an isolated entity whose mind is unaffected by the ongoing interaction;
3. The therapist pursues understanding in collaboration with the client, engaging the client as an equal partner, and in which the therapist's observations and interpretations are offered in a spirit of mutual discovery and inquiry—as possibilities rather than as objective pronouncements; and
4. The therapist is attentive to the pervasive mutual influences exerted in the therapeutic interaction, to the mutual evocation of emotional states and impressions, and is open to recognizing, acknowledging, and articulating his or her own role in creating conflicts and impasses.

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